EMERGENCY SURGERY · MIDTERM · COMPLETE QUESTION BANK

Emergency Surgery — Midterm All Lectures Question Bank

جراحة / طب الطوارئ — منهج الميدترم — مجمّع أسئلة المادة كلها

4	11	198	59
Lectures	Past exam questions	Expected Qs	Flashcards

#	Lectures included in this file · المحاضرات اللي في الملف	Past Qs	Expected	Cards
1	Acute Abdomen البطن الحادة	1	59	15
2	Acute Pancreatitis التهاب البنكرياس الحاد	4	39	14
3	Burn الحروق	2	50	15
4	Trauma الإصابات (Trauma)	4	50	15

المصادر:

كويز سابق (جراحة طوارئ)

ترقيم الأسئلة: رقم المحاضرة ثم رقم السؤال (مثلاً 4.12). الأسئلة المتوقعة عليها حرف N. الإجابات كلها في آخر الملف.

اللَّهُمَّ لا سَهْلَ إِلَّا ما جَعَلْتَهُ سَهْلًا، وأنتَ تَجْعَلُ الحَزْنَ إذا شِئْتَ سَهْلًا — صلِّ على النبي ﷺ

LECTURE 3

Burn

الحروق

Part A · Past Exam Questions

أسئلة الدفعات السابقة

3.1 According to the rule of nines, a 40-year-old patient who has burns over the front of chest and abdomen has about: كويز سابق (جراحة طوارئ)

a. 24% burns
b. 36% burns
c. 9% burns
d. 18% burns

3.2 The burn injury that occurs due to contact with strong acid or alkali is known as: كويز سابق (جراحة طوارئ)

a. Chemical burn
b. Inhalational burn
c. Thermal burn
d. Radiation burn

Part B · Expected Questions (from the lecture)

أسئلة متوقعة على المحاضرة بنفس أسلوب الكويزات

3.N1 By size and weight, the skin is the _____ largest organ in the body: MCQ

a. First
b. Second (after muscles)
c. Third
d. Fourth

3.N2 The epidermis is formed of: MCQ

a. Columnar epithelium
b. Stratified squamous epithelium
c. Muscle
d. Connective tissue

3.N3 Skin appendages include all EXCEPT: MCQ

a. Sweat glands
b. Sebaceous glands
c. Hair follicles
d. Lymph nodes

3.N4 Healing of second-degree burns occurs by epithelium creeping from the skin appendages. TRUE / FALSE

a. True
b. False

3.N5 All of the following are functions of the skin EXCEPT: MCQ

a. Formation of vitamin K
b. Barrier against micro-organisms
c. Prevents excessive water loss
d. Formation of vitamin D

3.N6 Scalds are caused by: MCQ

a. Electricity
b. Flame
c. Boiled liquids
d. Acids

3.N7 Flame burns are often due to: MCQ

a. Radiation
b. Hot liquids
c. Acids
d. Ignition of clothing

3.N8 Electric burns with high voltage may be fatal because: MCQ

a. They are superficial
b. They cause scalds
c. Blood and bones are good conductors
d. Skin is a good conductor

3.N9 Inhalation burns are due to exposure to: MCQ

a. Hot liquids
b. Hot gases
c. Sun
d. Acids

3.N10 The extent of burn is estimated by: MCQ

a. Parkland formula
b. Glasgow scale
c. Lund formula
d. Rule of 9

3.N11 A major burn involves more than _____ of body surface area: MCQ

a. 10%
b. 15%
c. 30%
d. 50%

3.N12 An intermediate burn in adults involves: MCQ

a. < 15%
b. 15–30%
c. 30–50%
d. > 50%

3.N13 A minor burn in children is less than: MCQ

a. 5%
b. 10%
c. 15%
d. 20%

3.N14 According to the rule of nines, each upper limb represents: MCQ

a. 1%
b. 9%
c. 18%
d. 27%

3.N15 According to the rule of nines, the perineum represents: MCQ

a. 1%
b. 9%
c. 18%
d. 4.5%

3.N16 The usual example of first-degree burn is: MCQ

a. Chemical burn
b. Scald
c. Sunburn
d. Electric burn

3.N17 Second-degree burns heal within _____ if no infection occurs: MCQ

a. 3 months
b. Never
c. 3 weeks
d. 3 days

3.N18 Infection of a second-degree burn may change it into a full-thickness burn. TRUE / FALSE

a. True **b.** False

3.N19 In third-degree burns there is: MCQ

a. Complete destruction of epidermis and dermis
b. Damage of epidermis and part of dermis
c. Erythema only
d. Damage of epidermis only

3.N20 Inhalation injury may lead to all EXCEPT: MCQ

a. Pneumonia **b.** Upper airway edema
c. Hypertension as the main problem **d.** Bronchospasm

3.N21 Neurogenic shock in burns is caused by: MCQ

a. Fluid loss **b.** Infection
c. Pain **d.** Heart failure

3.N22 Acute renal failure in burns occurs after: MCQ

a. Antibiotics **b.** Prolonged uncorrected hypovolemia
c. Skin grafts **d.** Excess fluids

3.N23 Curling's ulcer is: MCQ

a. Acute ulceration of the stomach and duodenum after burns
b. Corneal ulcer
c. Ulcer in an old burn scar
d. Pressure ulcer

3.N24 Burn wound infection usually occurs between: MCQ

a. Day 1–2 **b.** Days 4–7 postburn
c. 3 weeks **d.** 3 months

3.N25 The value of systemic antibiotics in prevention of burn wound infection is doubtful. TRUE / FALSE

a. True **b.** False

3.N26 Constricting eschars in deep circumferential burns of limbs and chest are treated by: MCQ

a. Urgent escharotomy **b.** Antibiotics
c. Skin graft **d.** Tracheostomy

3.N27 Edema in burns of the face and neck may need: MCQ

a. Urgent tracheostomy **b.** Escharotomy
c. Skin graft **d.** Fasciotomy

3.N28 **Marjolin's ulcer is:** MCQ

a. Acute gastric ulcer
b. Infected burn
c. Keloid
d. Malignant transformation in a long-standing burn scar

3.N29 **Local complications of burns include all EXCEPT:** MCQ

a. Compartment syndrome
b. Curling's ulcer
c. Contractures
d. Hypertrophic or keloid scar

3.N30 **First aid for burns includes pouring water over the burn for:** MCQ

a. 1 hour
b. 1 minute
c. 15 minutes
d. Ice-cold water for 30 minutes

3.N31 **Using ice-cold water in first aid of burns is contraindicated because it:** MCQ

a. Delays healing only
b. May induce more tissue damage
c. Causes infection
d. Is painful

3.N32 **Minor burns can be treated as outpatients.** TRUE / FALSE

a. True
b. False

3.N33 **Indications for hospital admission in burns include all EXCEPT:** MCQ

a. Chemical burns
b. Electric burns
c. Inhalation injury
d. First-degree sunburn of 5%

3.N34 **On admission of a burn patient, a Foley catheter is inserted to:** MCQ

a. Prevent infection
b. Measure CVP
c. Give drugs
d. Check urine output

3.N35 **The Parkland formula is:** MCQ

a. 2 mL/kg/% burn saline
b. 4 mL/kg/% burn lactated Ringer's per day
c. 4 mL/kg/% burn dextrose
d. 1 mL/kg/% burn plasma

3.N36 **According to Parkland formula, half of the amount is given in:** MCQ

a. The first 4 hours
b. The second day
c. The first 8 hours
d. The first 24 hours

3.N37 **In all fluid formulae, the maximum percentage of burn calculated is:** MCQ

a. 30%
b. 50%
c. 70%
d. 100%

3.N38 Oral intake in major burns is avoided during the first: MCQ

a. 6 hours
b. 24 hours
c. 48 hours
d. 1 week

3.N39 Adequate urine output during burn resuscitation is: MCQ

a. 0.1 mL/kg/hour
b. 0.5–1 mL/kg/hour
c. 3 mL/kg/hour
d. 100 mL/hour regardless of weight

3.N40 In burn patients, parenteral nutrition is preferred over enteral nutrition. TRUE / FALSE

a. True
b. False

3.N41 The three commonly used topical agents for burns are all EXCEPT: MCQ

a. Povidone iodine
b. Mafenide acetate
c. Silver nitrate solution
d. Silver sulphadiazine

3.N42 The ideal topical burn cream should be all EXCEPT: MCQ

a. Water-soluble base
b. Bactericidal
c. Toxic to viable cells
d. Not painful

3.N43 The exposure method of burn wound care is particularly indicated for burns of: MCQ

a. Feet
b. Circumferential limbs
c. Hands
d. Face, neck and perineum

3.N44 Advantages of the exposure method include all EXCEPT: MCQ

a. Avoids repeated dressing changes
b. No need for aseptic isolation
c. More comfortable
d. Dry air inhibits bacterial growth

3.N45 Partial-thickness skin grafts (Thiersch) are used to: MCQ

a. Cover deep face burns
b. Release eschars
c. Cover large raw areas produced by burns
d. Treat Curling's ulcer

3.N46 Full-thickness burns heal spontaneously within 2–3 weeks. TRUE / FALSE

a. True
b. False

3.N47 The most important prognostic factor in burns is: MCQ

a. Age
b. Site
c. Type
d. Extent

3.N48 The worst site of burns is the: MCQ

a. Hand
b. Perineum
c. Back
d. Face

3.N49 The major killer of burn victims is: MCQ

a. Contractures
b. Septicemia
c. Hypovolemia
d. Curling's ulcer

3.N50 The worst type of burn is: MCQ

a. High-voltage electric burn
b. Scald
c. Sunburn
d. Flame

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Skin functions	Barrier to microbes, prevents water loss, temperature regulation, sensation, vitamin D.	**Types of burn**	Scald (children), flame, electric (worst), chemical (acid/alkali), inhalation.
Extent	Major > 30%; intermediate 15–30% adults (10–30% children); minor < 15% adults (< 10% children).	**1st degree**	Epidermis only; erythema (sunburn); heals quickly.
2nd degree	Epidermis + part of dermis; blisters, painful; heals ~3 weeks if no infection.	**3rd degree**	Full thickness; leathery, painless; needs grafts.
Rule of nines	Head 9 · each arm 9 · each leg 18 · front trunk 18 · back trunk 18 · perineum 1.	**First aid**	Secure airway, strong analgesic, cool tap water 15 min (no ice).
Parkland	4 mL × kg × %TBSA Ringer's lactate: ½ first 8 h, ¼ second 8 h, ¼ third 8 h; max 50% TBSA in formula.	**Monitoring**	Urine output 0.5–1 mL/kg/h, vital signs, CVP in critical cases; NPO first 48 h.
Systemic complications	Inhalation injury, neurogenic & hypovolemic shock, renal failure, Curling's ulcer, MOF.	**Local complications**	Infection (4–7 days), constricting eschar, airway edema, compartment syndrome, contractures, Marjolin's ulcer.
Escharotomy	Release of constricting circumferential eschar in limbs/chest.	**Topical agents**	Silver sulfadiazine, silver nitrate, mafenide acetate; exposure or occlusive method.
Prognosis	Extent most important; face burns worst site; septicemia major killer; extremes of age worse.		

2.N32. a — Necrotizing pancreatitis (carbapenems preferred)

2.N33. c — ERCP

2.N34. b — Routine pancreatectomy for mild cases

2.N35. d — Inflammatory tissue (false cyst)

2.N36. b — 10% of cases
2–3 weeks after the attack.

2.N37. a — CT scan
Often painless and discovered on follow-up US.

2.N38. a — True

2.N39. c — 6 weeks
Internally into the stomach (cystogastrostomy) or jejunal loops.

3.1. d — 18% burns
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

3.2. a — Chemical burn
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

3.N1. b — Second (after muscles)

3.N2. b — Stratified squamous epithelium
Dermis = connective tissue.

3.N3. d — Lymph nodes

3.N4. a — True

3.N5. a — Formation of vitamin K

3.N6. c — Boiled liquids
Often affect children.

3.N7. d — Ignition of clothing

3.N8. c — Blood and bones are good conductors

3.N9. b — Hot gases
May affect upper or lower airway.

3.N10. d — Rule of 9
Modified in children (large head).

3.N11. c — 30%

3.N12. b — 15–30%
10–30% in children.

3.N13. b — 10%
< 15% in adults.

3.N14. b — 9%

3.N15. a — 1%

3.N16. c — Sunburn
Erythema only; heals rapidly.

3.N17. c — 3 weeks

3.N18. a — True

3.N19. a — Complete destruction of epidermis and dermis

3.N20. c — Hypertension as the main problem

3.N21. c — Pain

3.N22. b — Prolonged uncorrected hypovolemia

3.N23. a — Acute ulceration of the stomach and duodenum after burns
Rare now with antacids and H2 blockers.

3.N24. b — Days 4–7 postburn

3.N25. a — True
Prevention is by proper local wound care.

3.N26. a — Urgent escharotomy

3.N27. a — Urgent tracheostomy

3.N28. d — Malignant transformation in a long-standing burn scar

3.N29. b — Curling's ulcer
Curling's ulcer is SYSTEMIC.

3.N30. c — 15 minutes
Saline or tap water at room temperature.

3.N31. b — May induce more tissue damage

3.N32. a — True
< 15% adults, < 10% children: dressing + analgesia.

3.N33. d — First-degree sunburn of 5%
Also > 15% adults / > 10% children and burns with trauma or comorbidity.

3.N34. d — Check urine output

3.N35. b — 4 mL/kg/% burn lactated Ringer's per day

3.N36. c — The first 8 hours
1/4 in the second 8 hours, 1/4 in the third 8 hours.

3.N37. b — 50%
Otherwise serious over-infusion may occur.

3.N38. c — 48 hours

3.N39. b — 0.5–1 mL/kg/hour

3.N40. b — False
Enteral nutrition is preferred.

3.N41. a — Povidone iodine

3.N42. c — Toxic to viable cells

3.N43. d — Face, neck and perineum
And burns of one side of the trunk.

3.N44. b — No need for aseptic isolation
Exposure method requires isolation in a completely aseptic atmosphere.

3.N45. c — Cover large raw areas produced by burns
Full-thickness grafts for deep burns of face and hands.

3.N46. b — False
Partial-thickness heal in 2–3 weeks; full-thickness need skin grafts.

3.N47. d — Extent
Mortality ~50% if burn extent is 50%.

3.N48. d — Face
Associated inhalation injury.

3.N49. b — Septicemia

3.N50. a — High-voltage electric burn
Scalds with boiling water are the least harmful.

4.1. b — Penetrating trauma
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

4.2. b — (ABCDE) assessment
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

4.3. d — Cardiac tamponade
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

4.4. a — Needle decompression
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

4.N1. b — Exchange of energy beyond the body's resilience

4.N2. b — 1–44
3rd leading cause for all ages.

4.N3. c — 3rd

4.N4. c — Don't get distracted with ugly injuries; initial assessment determines how to proceed

4.N5. b — The right patient gets to the right hospital in the right time

4.N6. b — Ankle fracture

4.N7. c — The person continues moving forwards and the head strikes the car

4.N8. a — High velocity injuries
Knives and pistols are low velocity.

4.N9. c — K = MV/2g ($MV^2/2g$)
Velocity has the largest effect.

4.N10. a — Secondary missiles

4.N11. d — Minor trauma
Minor: single part/system, e.g. small 2nd-degree burn, cut requiring stitches.

4.N12. c — Major trauma
Serious single-system injury.

4.N13. c — Motor vehicle accidents

4.N14. b — Heart rate

4.N15. b — 0–4
Total RTS 0–12.

4.N16. c — 11

4.N17. c — Colored labels according to injury and resources availability

4.N18. d — Will survive anyway
Lecture: Red = die anyway, Yellow = survive only with medical attention, Green = survive anyway.

4.N19. c — Will survive only if receiving medical attention

4.N20. c — Major trauma
Primary survey + resuscitation → secondary survey → definitive treatment.

4.N21. a — ABCDE

4.N22. b — Airway with cervical spine protection

4.N23. c — What is your name / can the patient talk?

4.N24. c — Needle decompression
Needle decompression is a BREATHING intervention.

4.N25. c — Isolated finger injury
Also maxillofacial trauma.

4.N26. b — Rigid collar and back board or manual in-line immobilization

4.N27. b — False
Airway patency does NOT ensure adequate ventilation.

4.N28. b — Crepitus (surgical emphysema)

4.N29. a — Needle decompression

4.N30. a — Occlusive dressing

4.N31. a — Chest tube

4.N32. d — Blood sugar

4.N33. a — True

4.N34. b — Obstructive
Obstructive: physical obstruction reduces cardiac output.

4.N35. a — Disruption of vasomotor tone

4.N36. b — Oral fluids

4.N37. a — 0.5–1 mL/kg/hour

4.N38. d — Disability (brief neurological exam)
GCS is performed during the secondary survey.

4.N39. b — Response to painful stimuli